Patient journey and wireframes

From "I need a doctor" to "I know what comes next". One patient's route through Carevia, shown on desktop web and mobile, with a list of what we improved. This revision matches the working demo: six booking steps, a consent step, a "confirm it's you" step before the call, moving a visit, the doctor's screens, and an iMED status after the visit instead of documents.

INSIDE

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SECTION 1

How to read this document

These are wireframes, not final designs. They show what goes on each screen and in what order: the content, the buttons and the steps. Colours, photos, fonts and final wording come later. Grey boxes with a cross are placeholders for images.

Prices and names are examples. Prices come from the current website and are confirmed before launch. Carevia serves patients in mainland Portugal. "Clinician name" is a placeholder.

Numbered black dots on a screen match the notes beside it. Each note says what that part does, and whether it is new or changed.

Solid frame: this screen exists on today's website

Dashed frame: a new screen we are proposing

CHANGED — Something that works differently from today

NEW — Something that doesn't exist today

REVISED — Changed after the build, November 2026 (see section 3b)

1 — A numbered note, explained beside the screen

The journey at a glance

Five stages. Today's website covers the first stage. Stages two to five are promised by the website's own words ("Your first visit, step by step") and are designed here.

1	2	3	4	5
Discover	**Decide**	**Book**	**Visit**	**After**
Land on the website. Search a symptom or browse the seven services.	Choose the care. Check who it's for and that you're in mainland Portugal. Pick a doctor and a time.	Create an account or sign in. Read and accept the consent. Review the visit and pay.	Prepare, confirm it's you, test the camera, wait a moment, then talk by secure video.	See the visit's status. Your record is in iMED. Book a follow-up.
SCREENS 01–03	SCREENS 04–05	SCREENS 06–07	SCREENS 08–10	SCREENS 11–12
○ Worried, unsure	● Reassured	○ Most effort	○ Nervous, then relieved	● In control

SECTION 2

The journey map

What the patient wants, does and feels at each stage, and where things could go wrong.

	1 · Discover	2 · Decide	3 · Book	4 · Visit	5 · After
THEY WANT	"Can someone help with this, without a waiting room?"	"Is this the right service, and what will it cost?"	"A time with someone I trust, for me or my child."	"To be heard, and leave knowing what to do."	"To know what happens next, and where my record is. A follow-up if I need one."
THEY DO	Land on the homepage. Search a symptom ("fever", "back pain") or browse care.	Open a service. Read the price, the length and what it covers. Confirm who it's for and that they'll be in mainland Portugal.	Pick a doctor, language and time. Create an account. Accept the consent. Pay.	Follow a short checklist. Confirm email and mobile. Test camera and mic. Join the video call.	See the visit's status. Find the clinical summary, and anything the doctor decided to issue, in iMED. Book again.
THEY FEEL	○ Worried, a little sceptical	● Reassured by clear prices	○ The hardest part: forms and payment	○ Nervous before, relieved after	● In control
WHERE	Homepage, search, care list	Service details, booking steps 1–3	Booking steps 4–6	Visit page, "confirm it's you", waiting room, video call	Patient account
COULD GO WRONG	Someone with an emergency uses the site instead of calling 112	Patient picks a service, then has to pick it again	Patient finds out they're not eligible only after starting to pay	The video connection fails	The clinician says the issue needs an in-person visit
HOW WE HANDLE IT	Emergency message on the service details and in booking	The chosen service carries through to booking	Age and place (mainland Portugal) are checked before any details	Reconnect while the visit is open, otherwise move the same appointment to a new time	The status says in-person care is advised. The doctor's reasons are in iMED

SECTION 3

What we changed, and why

Compared with today's website and the original plan. Each change has the same aim: fewer surprises, and fewer steps between "I need help" and "I'm talking to a doctor".

#	Before (today's website)	After (this design)	Why it's better
1	The service details window says "Explore on Carevia". It opens the general services page, so the patient has to choose again.	The button says "Book General care" (or whichever service) and opens booking with that service already chosen.	One less decision. The patient doesn't lose their place.
2	"Find your care", "Browse our care" and the footer button all scroll down to the same list. No button starts a booking.	Browsing still scrolls to the list. Booking starts from any service's details. Returning patients get "Book new care" in their account.	Clear difference between "look around" and "book now".
3	The website has "Create your account" but no way to sign in.	"Sign in" sits in the top bar. Returning patients go straight to their account home (screen 12).	Returning patients don't have to start from the homepage.
4	The 18+ rule and the place limit are only in the FAQ answers.	Booking asks "Who is this visit for?" and "Will you be in mainland Portugal during the visit?" before any details.	Nobody finds out they can't book after giving their details or card.
5	"Call 112" is in the FAQ and the service details note.	It is also on booking step 1 and in the waiting room.	The message appears where someone in a hurry will see it.
6	No booking flow existed, so the original plan had no rule for when to create the account.	The patient chooses a clinician and time first. The account comes after, and the time is held for 10 minutes.	People see real availability before being asked for personal details. More people finish booking.

What we changed, and why

#	Before (today's website)	After (this design)	Why it's better
7	"Pay per visit, no subscription" and what happens if the video fails are in the FAQ and the "Your first visit" section.	Both are repeated on the payment screen, right beside the Pay button: "If the video fails, reconnect, then reschedule if needed. Same appointment."	Reassurance appears at the moment of doubt.
8	Preparation tips are written as a short article.	The tips become a checklist on the confirmation screen, with a "Test camera and mic" button.	Easier to act on. Technical problems are caught before the call, not during it.
9	Nothing between booking and the call.	A waiting room shows camera, mic, speaker and connection status, and how long the visit stays open if the connection fails.	Calms the most anxious moment.
10	Patients can't choose a time and a clinician on the website.	A "First available" option, plus filters for day and language.	The fastest route for urgent needs, and choice for people who want it.
11	"Your care in one place" was a promise only.	A visit status page (screen 11) and an account home (screen 12) with visits and family members. The clinical record lives in iMED, not in Carevia.	The patient always knows what happened and where to look.
12	Children and dependants are mentioned only in the FAQ.	"Me / My child / Another adult I care for" in booking step 1, and a Family section in the account.	Parents can book for a child without a workaround.
13	Seven service cards. A patient who isn't sure which fits has no way in.	An eighth card: "Not sure which one?", which starts with General care.	Undecided patients still have an easy first step.

Web compared with mobile

Booking summary on the side
On web, your service, time and price stay visible on the right through every booking step. On mobile they appear on the review screen.

Service details as a window
Web shows details in a centred window with a photo. Mobile slides up a panel from the bottom.

Your questions beside the video
On web, the questions you wrote before the call sit beside the video. Mobile keeps the screen for the call only.

Account menu on the left
Web has a left menu (Home, Family, Profile, Security). Mobile uses a compact menu at the top.

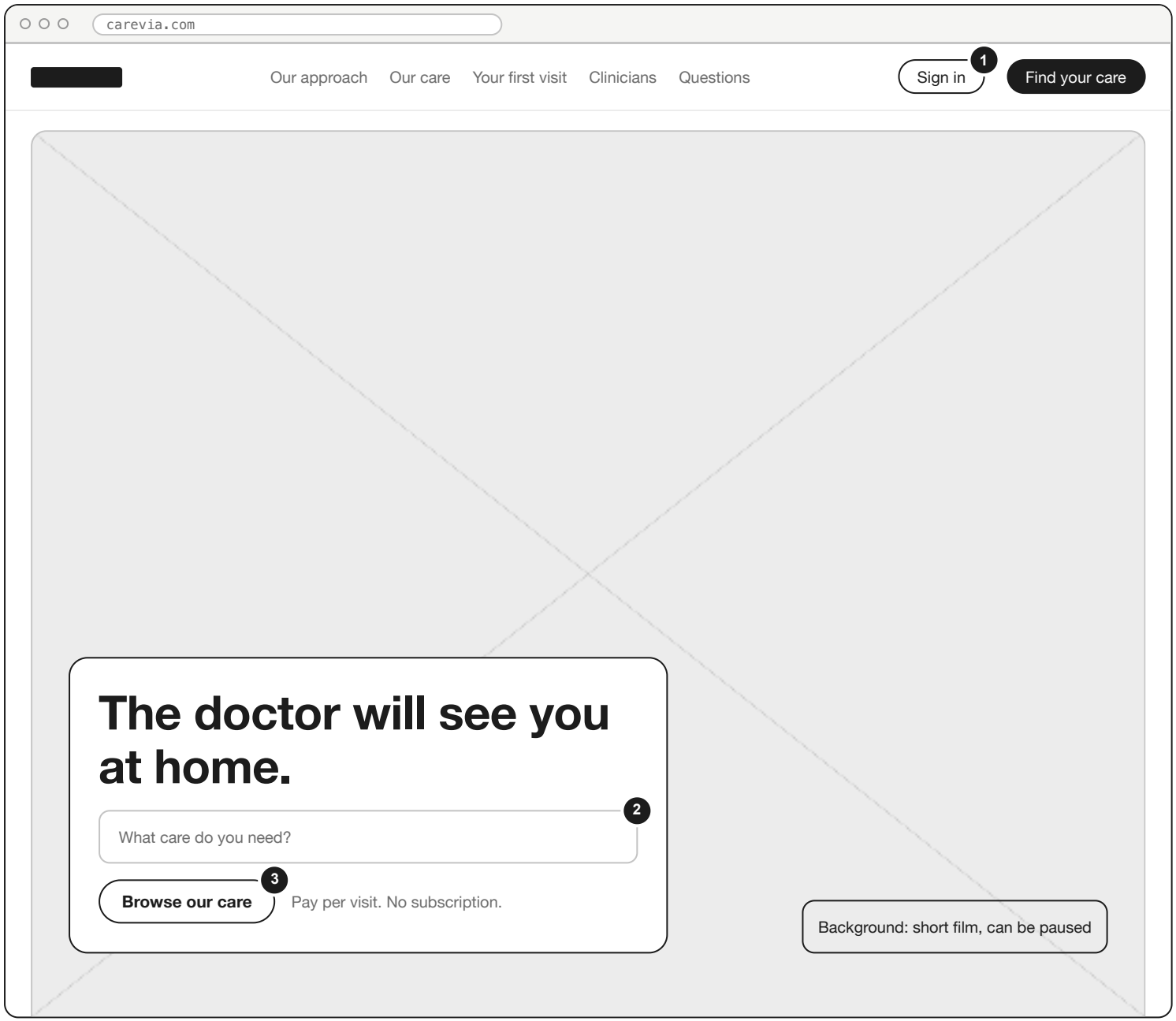
What changed after the build

Building the demo against Portuguese telemedicine rules and iMED changed these screens. Where the first draft and the rules disagreed, the rules won. Pages marked REVISED show the new version.

#	First draft	Now (working demo)	Why
1	Four booking steps.	Six: care and type, who and where, doctor and time, your details, consent, review and pay.	Consent is required before a visit, and the care is confirmed in booking itself (screens 03b, 06b).
2	A country list.	"Will you be in mainland Portugal during the visit?" Yes or no.	Carevia serves mainland Portugal only. Madeira and the Azores have their own regulators.
3	Clinician cards with a name and languages.	Each card also shows the Ordem dos Médicos cédula and the doctor's fee.	Patients must see who the doctor is and that they are registered.
4	Account: full name, email, password, date of birth.	First and last name, email, mobile, date of birth, password. A child is chosen or added in the next step.	The mobile is confirmed before each visit. No NIF, ID number or address at sign-up.
5	"Free rebook if the video fails."	Rejoin while the visit is open; otherwise move the same appointment to a new time (screen 08c).	A dropped call never creates a second booking or a second charge.
6	Card fields on the payment screen.	"Pay (demo)" with no card fields. A real provider comes later.	The demo takes no money. A visit is confirmed only after payment.
7	Straight from booking to the waiting room.	"Confirm it's you": the email and mobile for the visit, and an optional companion (screen 08b).	Portuguese teleconsultation practice; the right to be accompanied (Lei 15/2014).
8	Screen 11: a summary, next steps and downloadable documents.	Screen 11: the visit's status and an iMED explanation. Nothing to download in Carevia.	Prescriptions, sick-leave certificates (CIT), certificates and referrals exist only in iMED, the certified clinical system.
9	Account menu: Home, Visits, Documents, Family, Settings.	Home, Family, Profile, Security.	Carevia keeps no clinical data or documents.
10	"Your questions" typed in a form.	The same list, saved only on the patient's device.	No symptoms or clinical notes are stored by Carevia.
11	Reminders 24 hours and 15 minutes before.	A calendar file and the confirmation email; reminders wait for real email and SMS.	No real email or SMS is sent in the demo.
12	No doctor screens.	An agenda, the visit room (the doctor opens the call) and four outcome buttons (section 4b).	Reasons and notes go in iMED; Carevia only records what happened to the appointment.

01 · Homepage

EXISTS TODAY, SMALL CHANGE

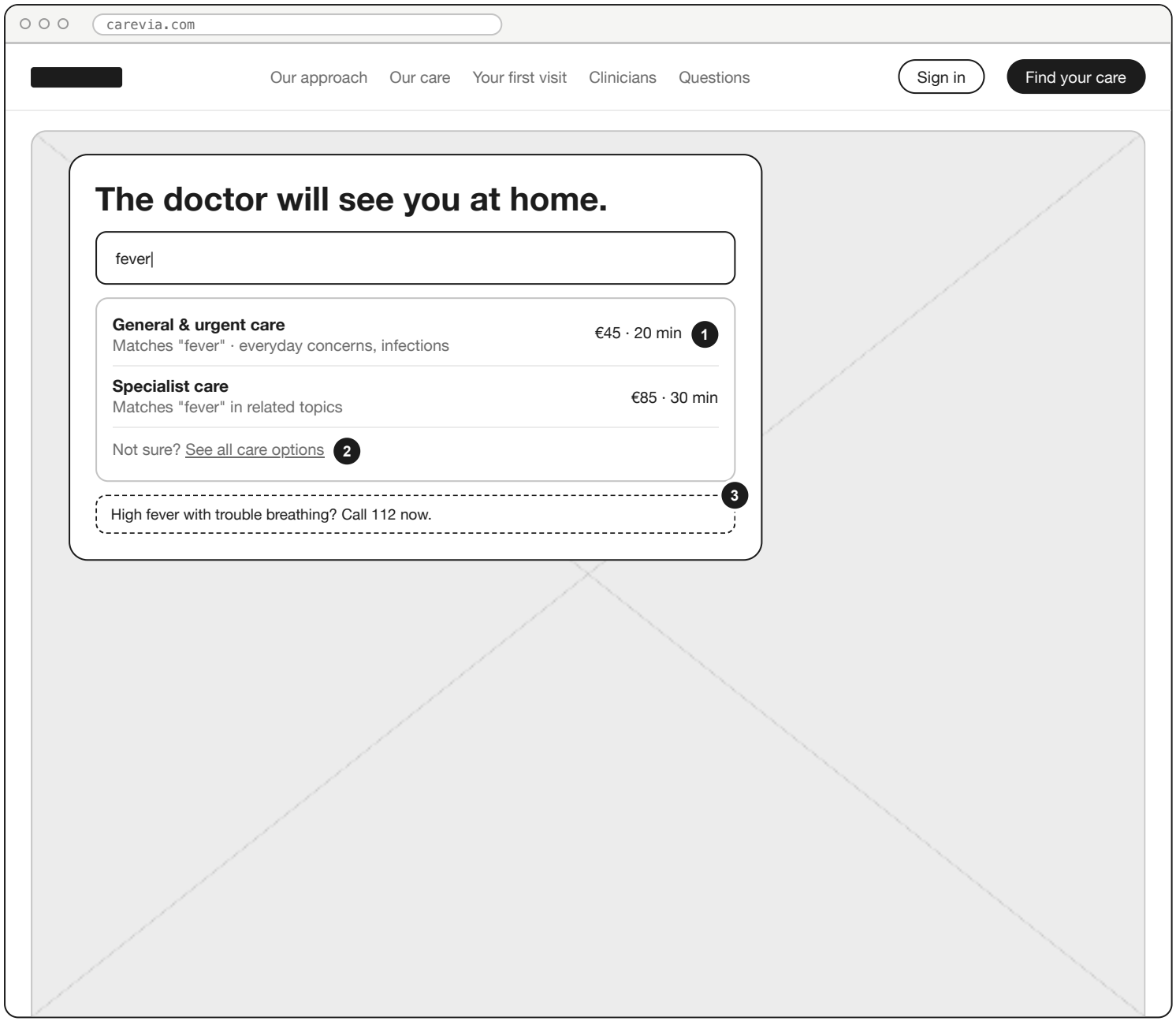


1. **Sign in** NEW
   Returning patients go straight to their account (screen 12). Today the site can't take them there.
2. **Search by symptom**
   People type what's wrong ("fever", "sick leave") and get the right service. See 01b.
3. **Browse our care**
   Scrolls to the list of services (screen 02). Unchanged.

Goal of this screen: in five seconds, the visitor knows Carevia is a real doctor by video, at home, with clear prices.

01b · Searching by symptom

EXISTS TODAY

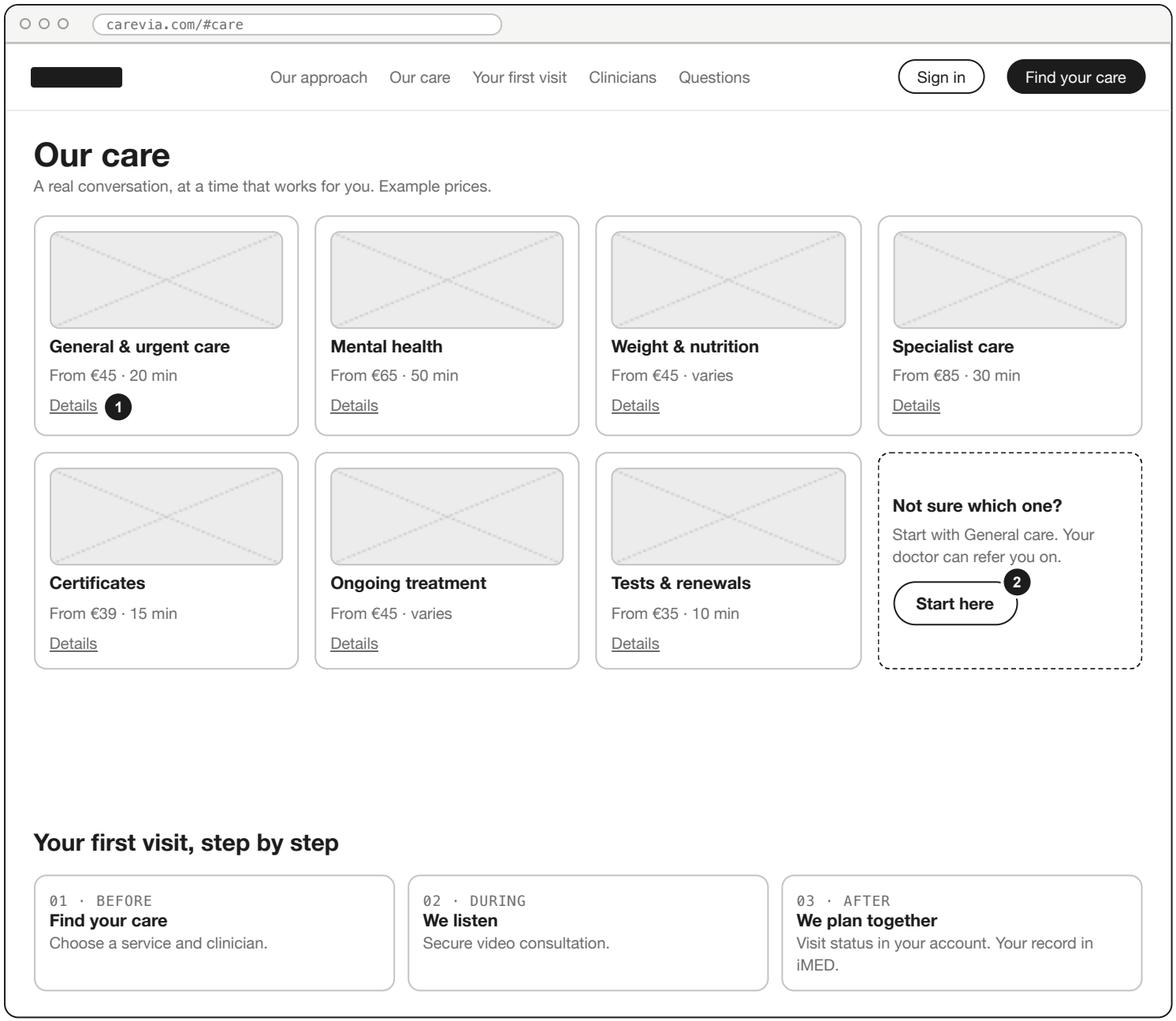


1. **Best match first**
   Results show price and length. Choosing one opens the service details (screen 03).
2. **Way out**
   If nothing fits, jump to the full list.
3. **Emergency hint** NEW
   Shown when the search sounds urgent. Wording to be agreed with your clinical team.

Why search matters: patients know their problem, not the department. "Back pain" finds General care; "driving" finds Certificates & assessments.

02 · Our care

EXISTS TODAY, ONE ADDITION



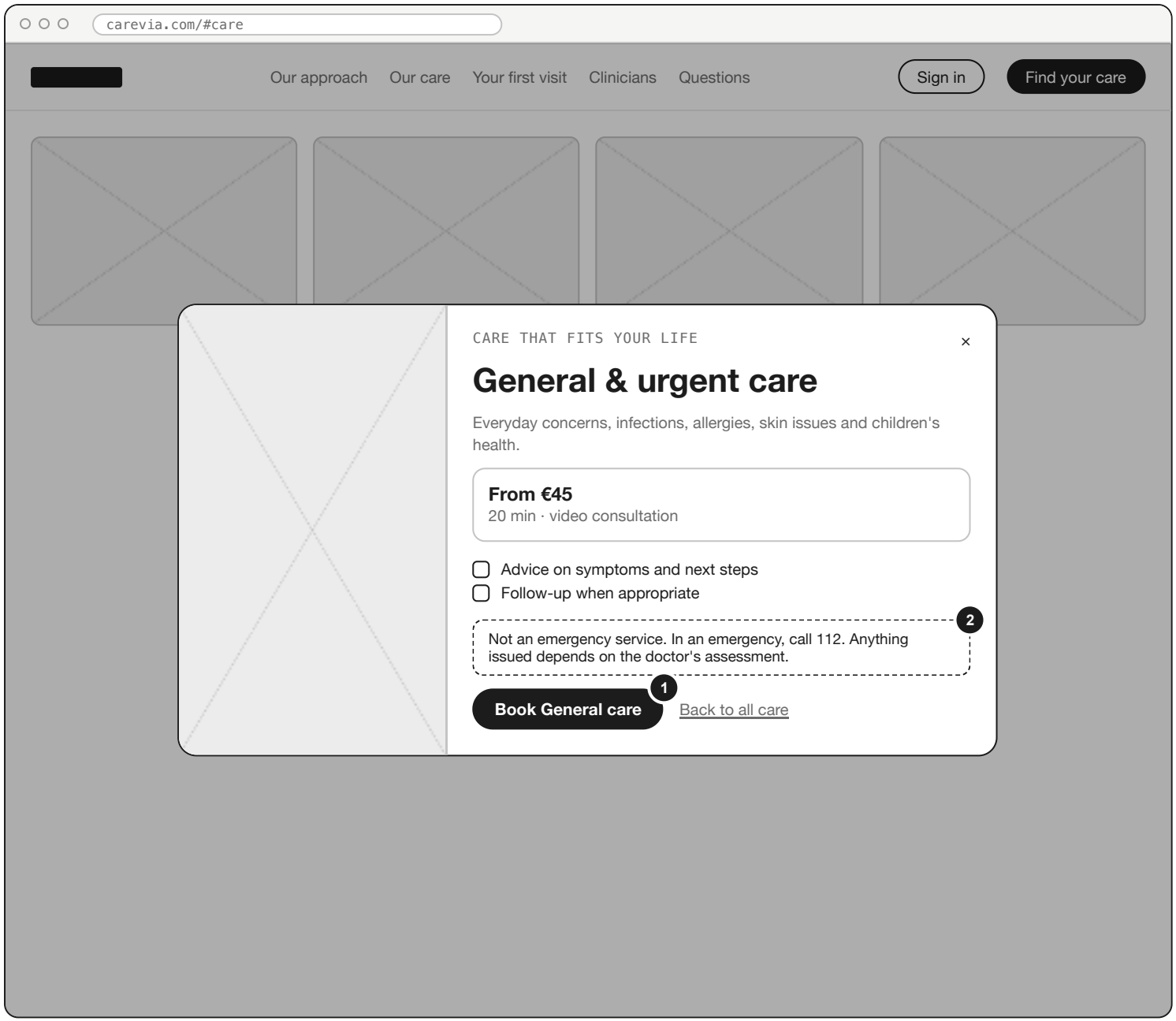
1 **Details**
Every card shows its price and length up front. Details opens the service window (screen 03).

2 **"Not sure which one?"** NEW
A friendly start for undecided patients. Goes to booking with General care chosen.

Goal: compare all care side by side, with nothing hidden. Prices are examples and are confirmed before launch.

03 · Service details

EXISTS TODAY, CHANGED



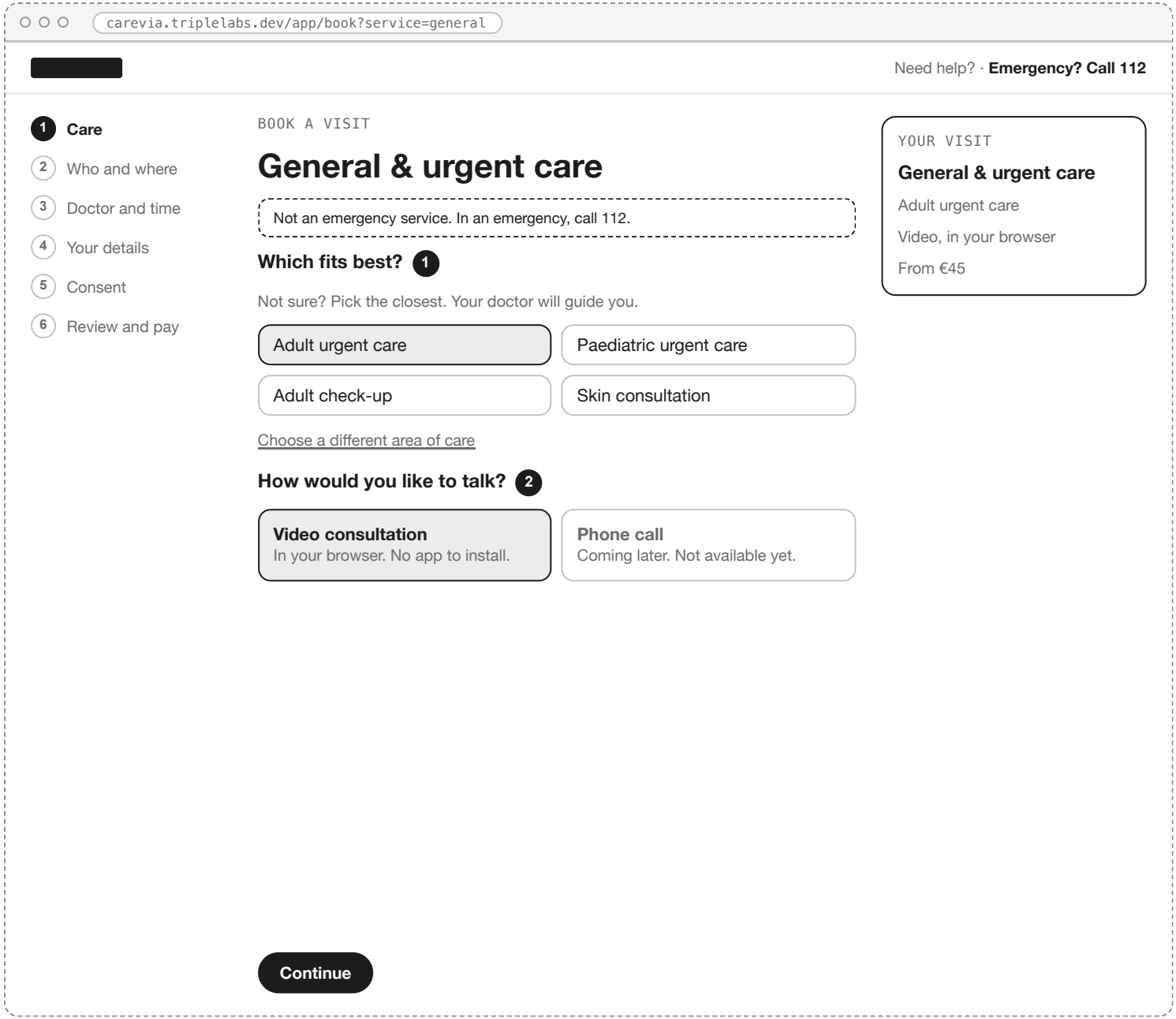
1. **"Book General care"** CHANGED
   Today this button says "Explore on Carevia" and opens a general page, so the patient chooses again. Now it opens booking with this service already chosen.

2. **Emergency note**
   Kept from today, now with the number to call.

This is the decision point. Everything the patient needs to say yes is in one window: what it covers, how long, how much.

03b · Booking step 1: care and consultation type

NEW SCREEN

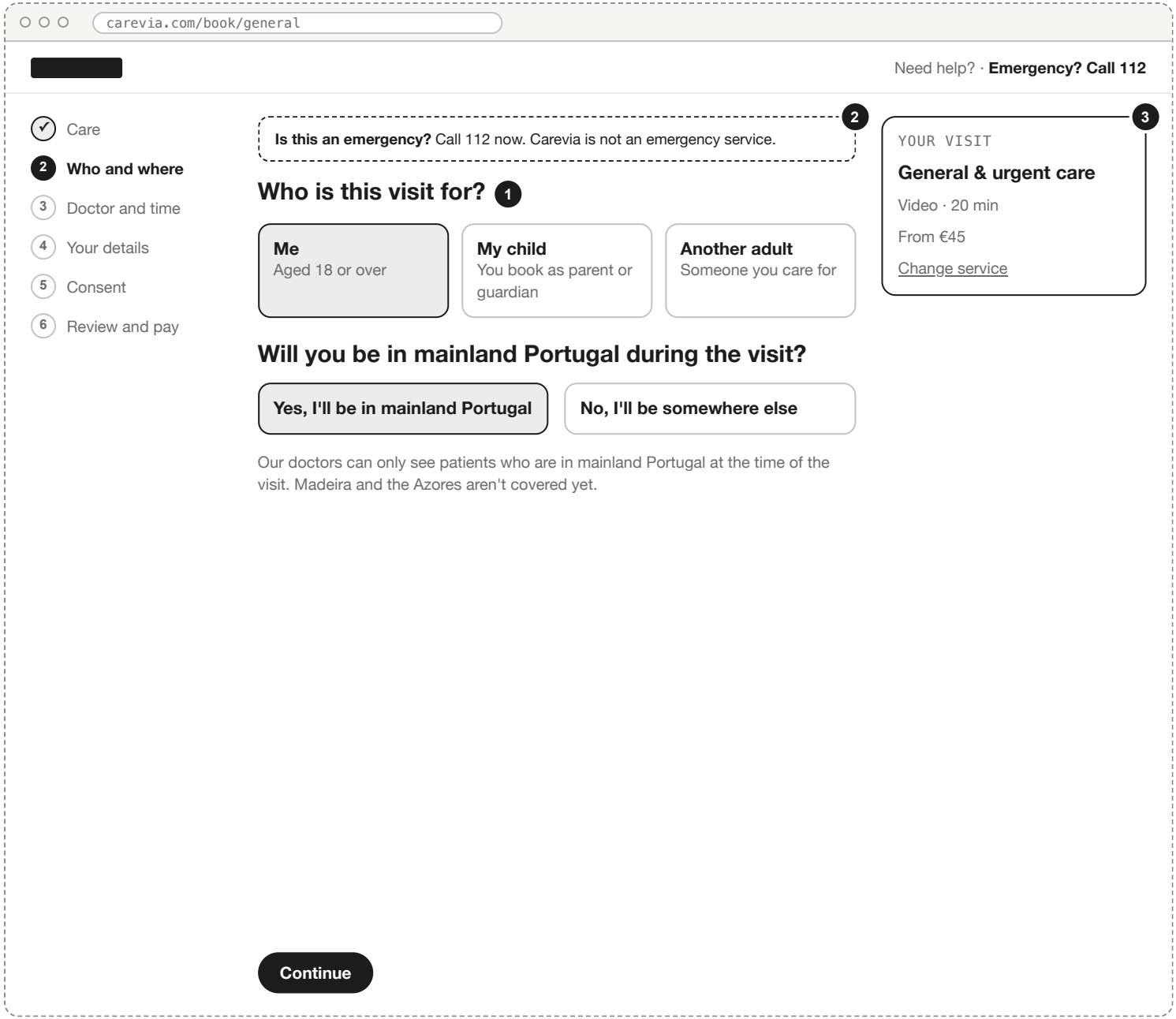


1. **The care, chosen once** NEW
   Opened from "Book General care" on screen 03 with the area already chosen; the patient picks the closest service.
2. **Video or phone** NEW
   The consultation type decides the rest of the flow. Video first; phone calls to a confirmed mobile come later.
3. **Certificates and test requests** NEW
   For services named after a document, this step also says the doctor decides after assessing, and booking doesn't guarantee a certificate, a test request or a renewal.

Arrives here from: the homepage's "Book {service}" and "Not sure which one?" (General care).

04 · Booking step 2: who and where

REVISED

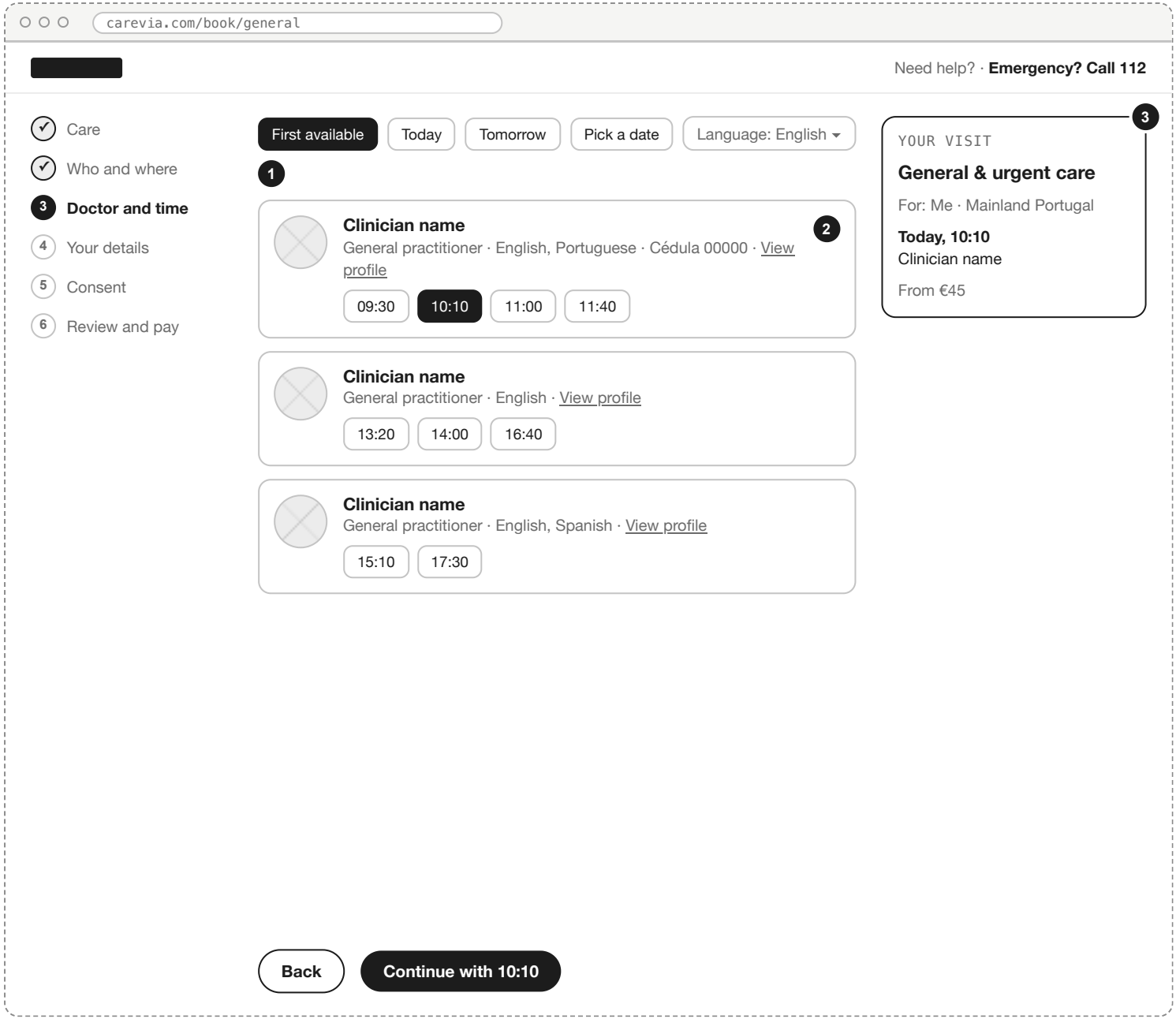


1. **Who and where, first** REVISED
   A yes-or-no question replaces the country list: Carevia serves mainland Portugal only. "No" explains why and stops here, before any details or payment.
2. **Emergency reminder** NEW
   Placed where someone in a hurry will see it.
3. **Visit summary** NEW
   Stays on the right for all six steps, so the patient always sees what they're booking and the price. Web only.

Arrives here from: step 1, where the patient confirms the care and chooses a video consultation (phone calls come later). "Book General care" on screen 03 opens step 1 with the service already chosen.

05 · Booking step 3: doctor and time

REVISED

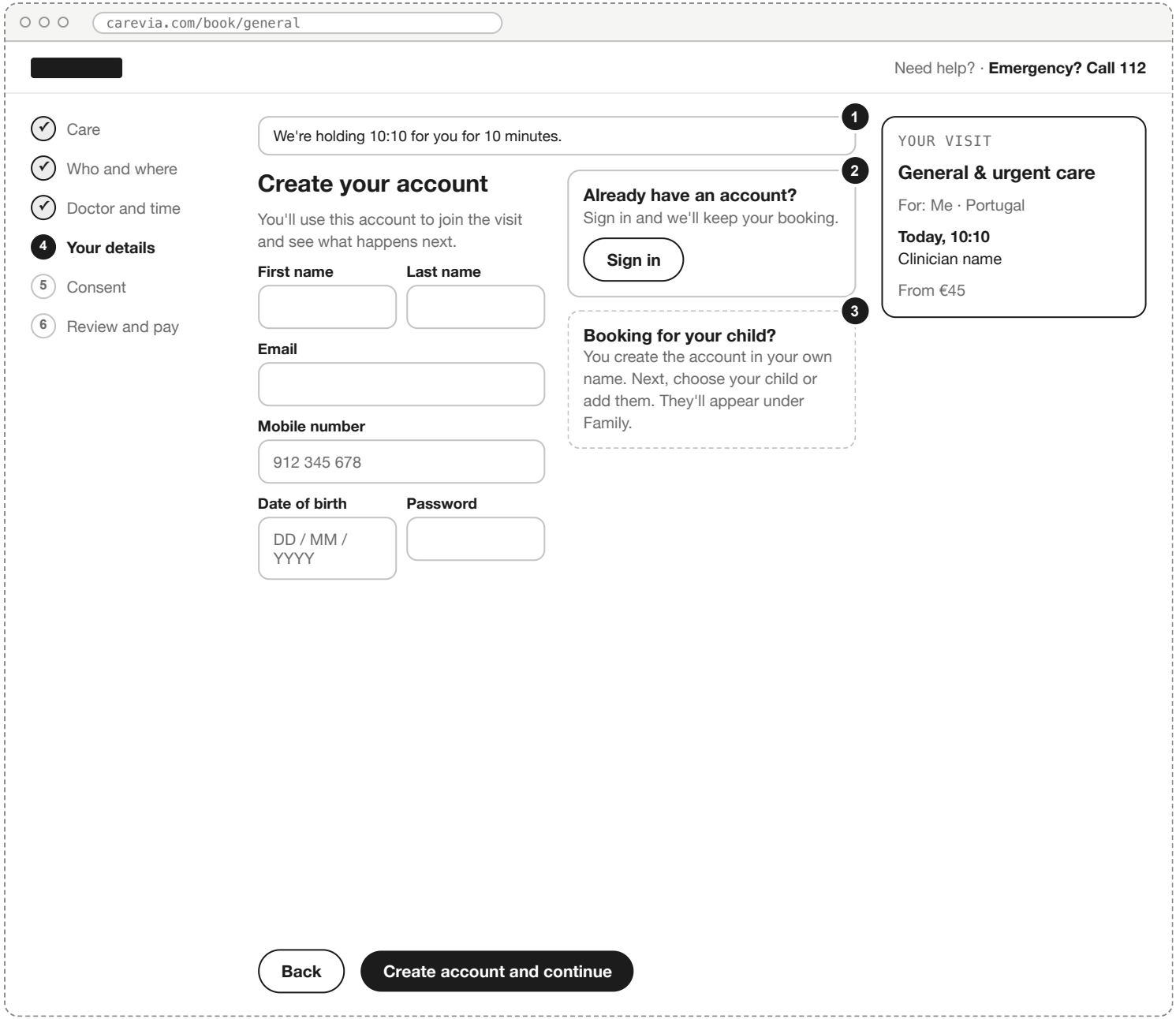


1. **First available, day and language** NEW
   "First available" is the fastest route for urgent needs. Language matters for being heard.
2. **Clinician with times**
   Pick a person, or just a time. Every card shows the doctor's Ordem dos Médicos cédula and their fee; "View profile" adds experience and languages. Photos are labelled "Illustrative" in the demo.
3. **Summary updates**
   The chosen time appears straight away.

Matches the website's promise: "Check the language, availability, and price before you book."

06 · Booking step 4: your details

REVISED

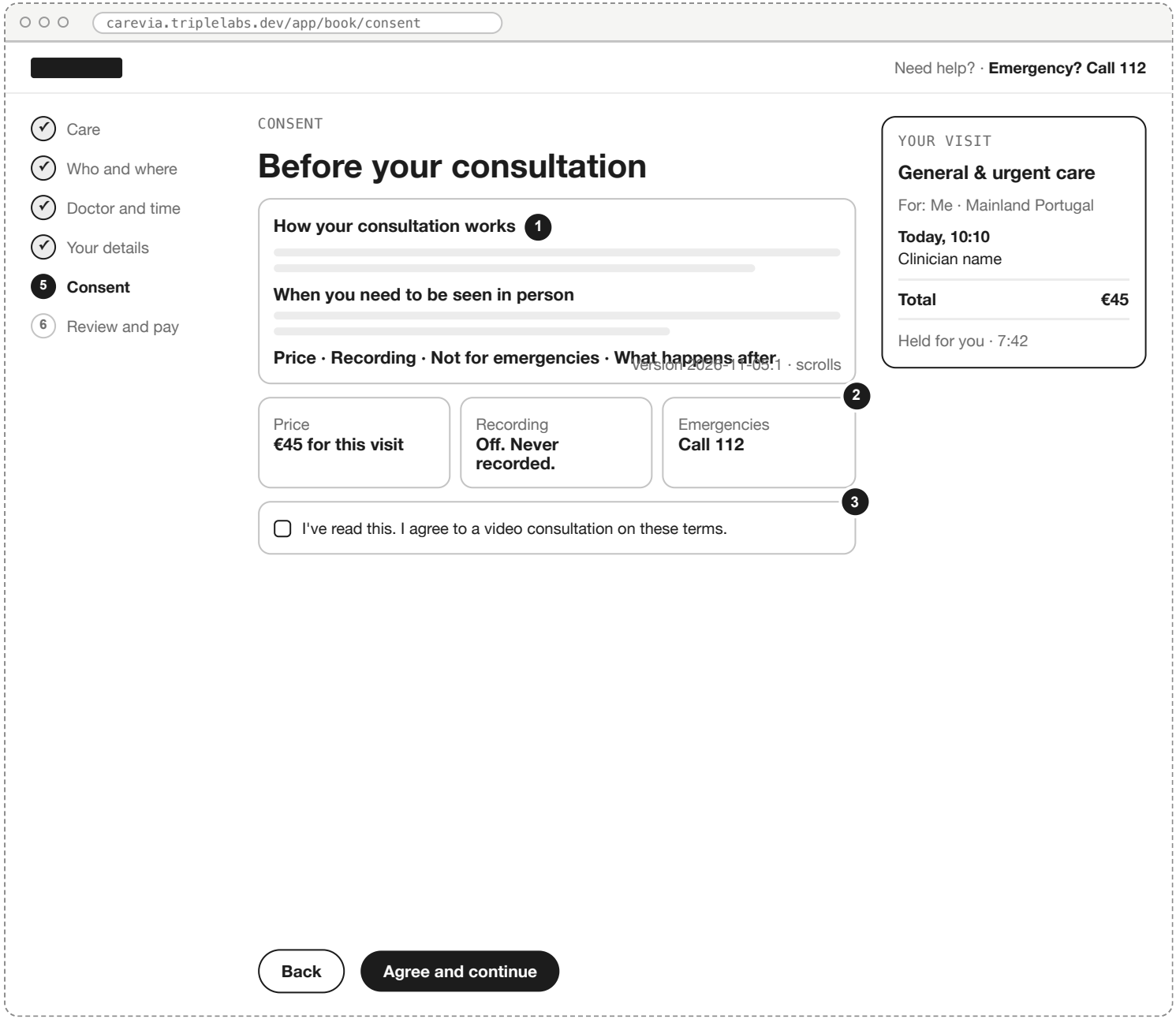


1. **Time held while you sign up** NEW
   The account comes after choosing a time, not before. People see availability first, so more of them finish.
2. **Sign in instead**
   Returning patients skip the form.
3. **Children and dependants** NEW
   A parent or guardian books on their behalf, as the FAQ promises.

Kept short on purpose: name, email, mobile, date of birth and password. No NIF, ID number or address. Medical history is shared with the doctor in the call, never in a Carevia form.

06b · Booking step 5: consent

NEW SCREEN

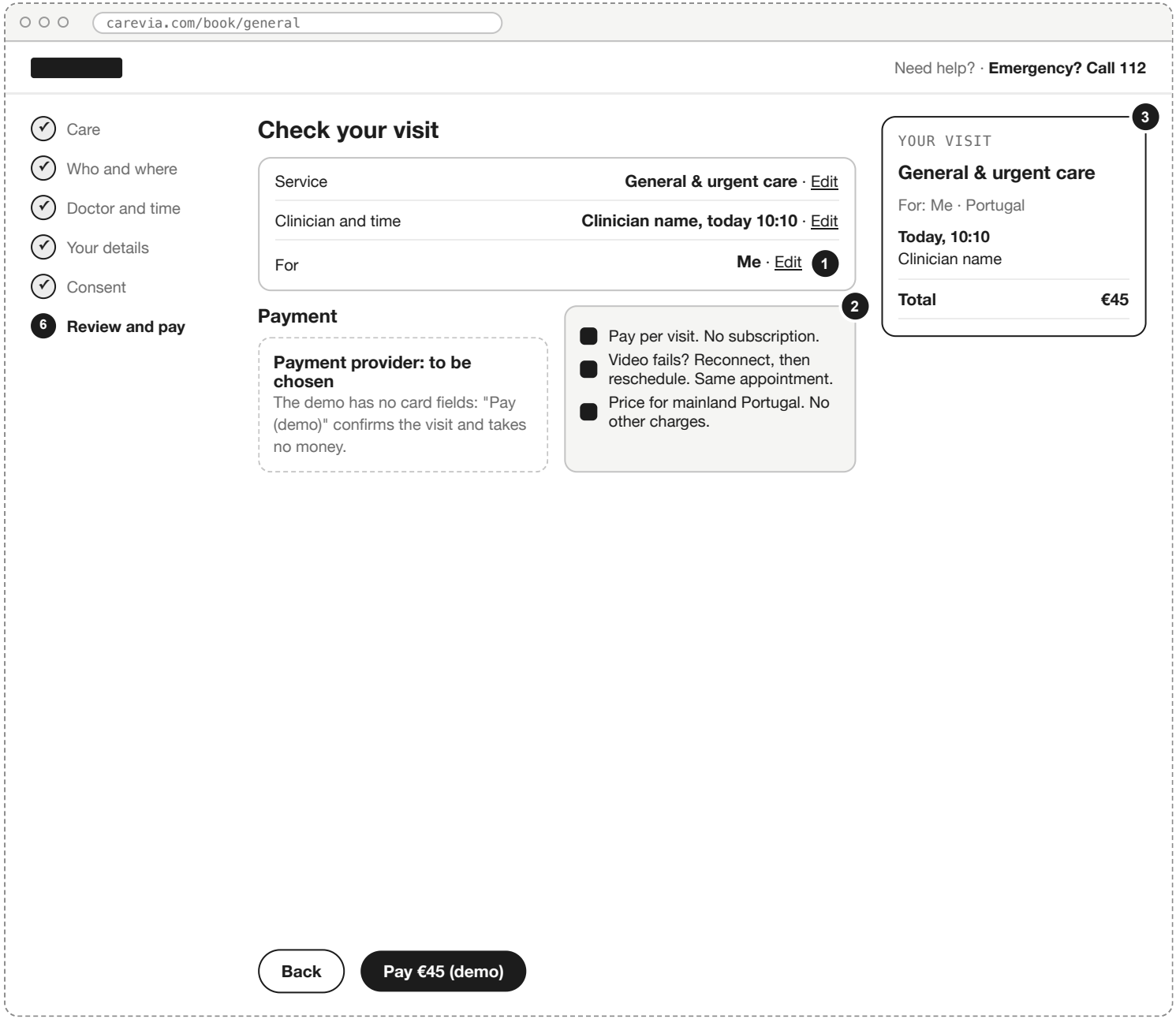


1. **What the patient agrees to** NEW
   How care is delivered, that in-person care is the alternative, the price, that nothing is recorded, the 112 limit, and that the doctor decides what to issue. The text is a draft for Carevia's lawyer.
2. **The three facts that matter most**
   Repeated as tiles so nobody misses them in the long text.
3. **One tick, stored**
   Carevia stores which version was accepted and when. For a child or another adult, the box says "on behalf of" them.

Required before the visit by Portuguese teleconsultation rules. Recording is never offered in this build, so there is no separate recording consent.

07 · Booking step 6: review and pay

REVISED

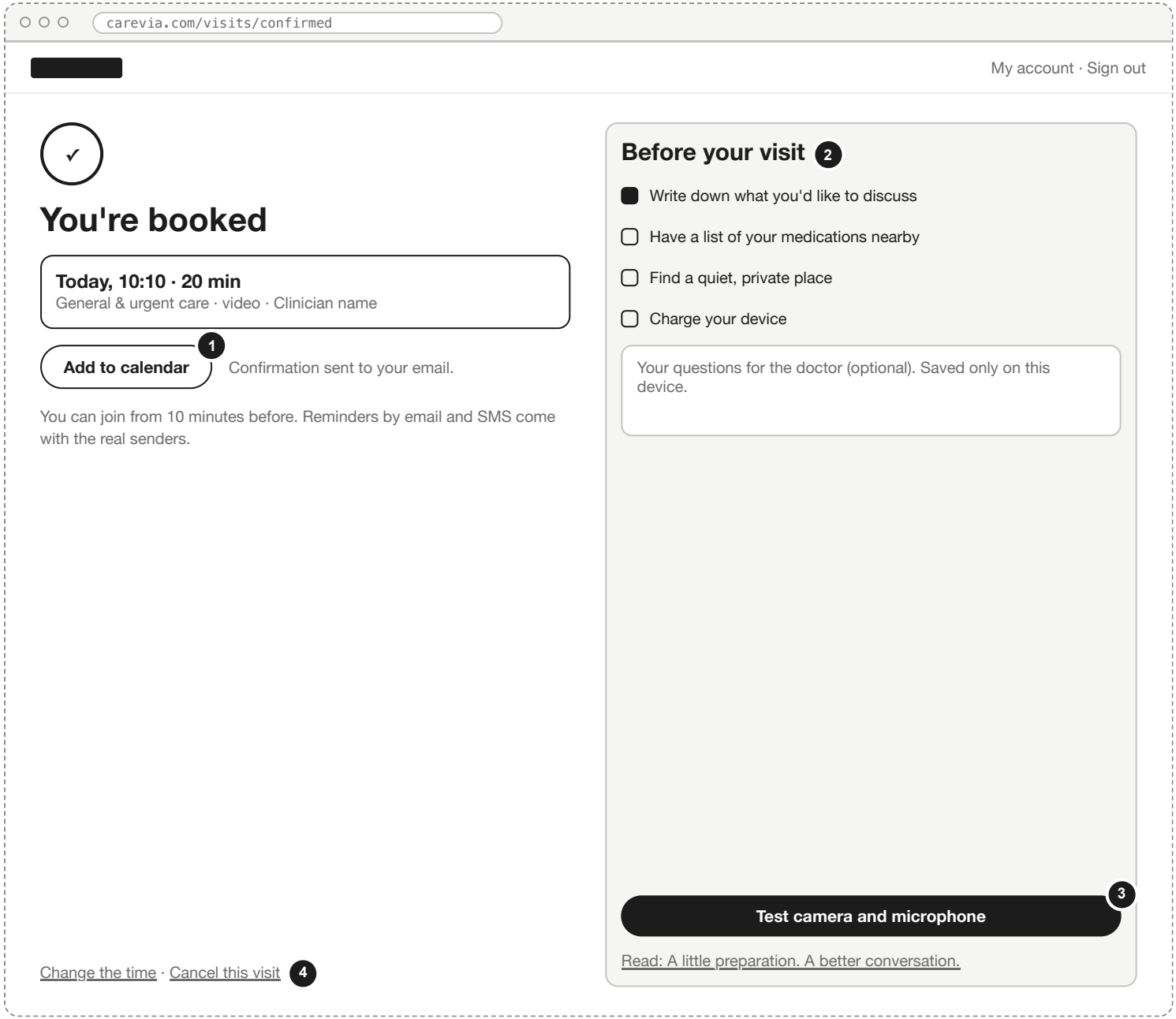


1. **Edit anything**
   Go back to any step without losing the rest.
2. **Reassurance beside the Pay button** CHANGED
   Today these promises are in the FAQ. Now they appear where people hesitate.
3. **Final price**
   The doctor's fee for this visit. No hidden fees. The visit is confirmed only after payment.

Before this step: the consent (screen 06b). **Open question:** which payment provider, and whether insurance or vouchers are needed. See the last page.

08 · You're booked, and how to prepare

REVISED

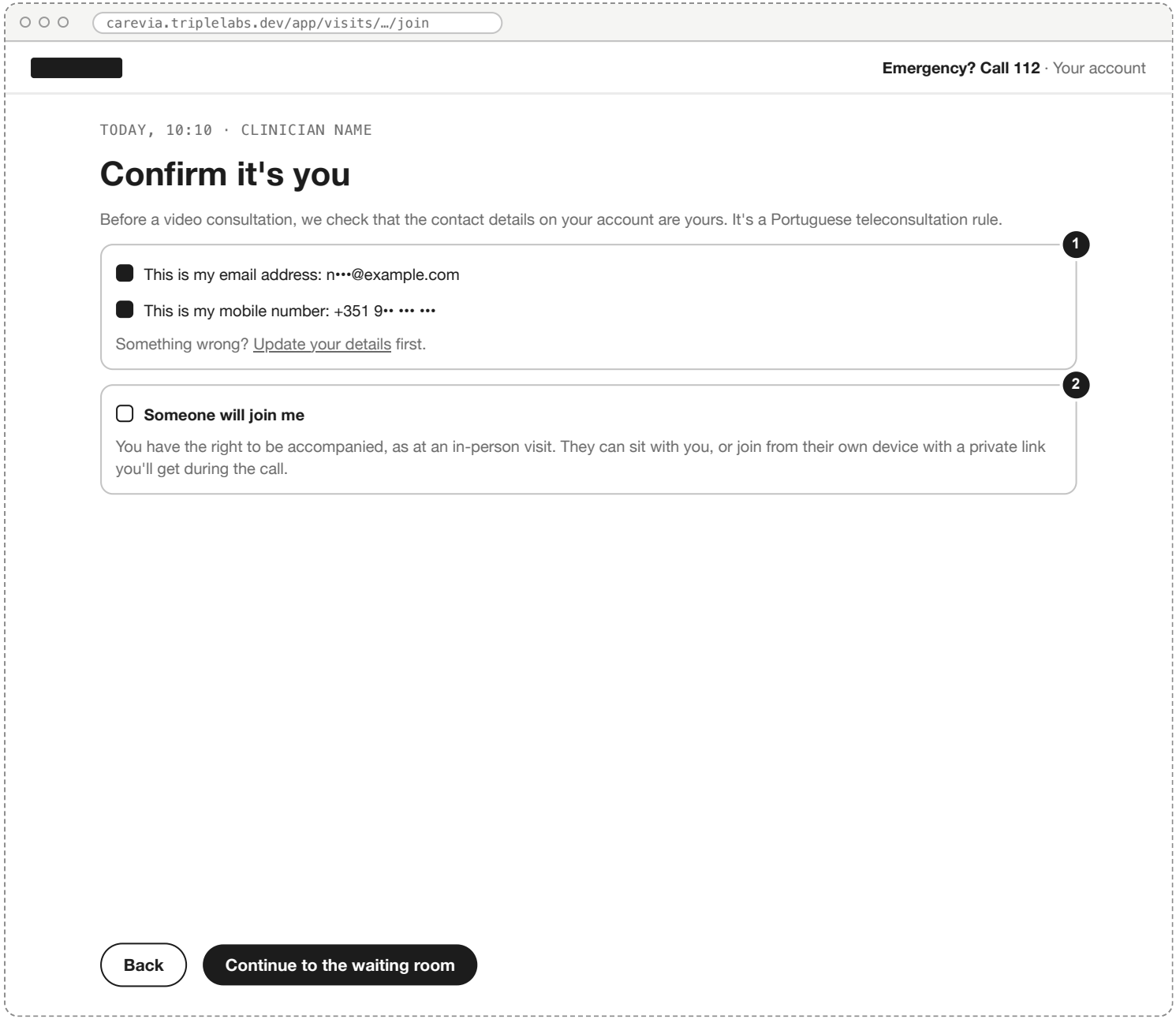


1. **Calendar and reminders**
   Fewer missed visits.
2. **Preparation checklist** CHANGED
   Today's preparation article, turned into ticks. Questions typed here reappear beside the video (screen 10).
3. **Test camera and mic now** NEW
   Problems are found before the visit, not during it.
4. **Change the time** NEW
   Moves the same appointment (screen 08c). The questions never leave the patient's device.

Goal: turn waiting time into preparation, so the 20 minutes are used well.

08b · Confirm it's you

NEW SCREEN

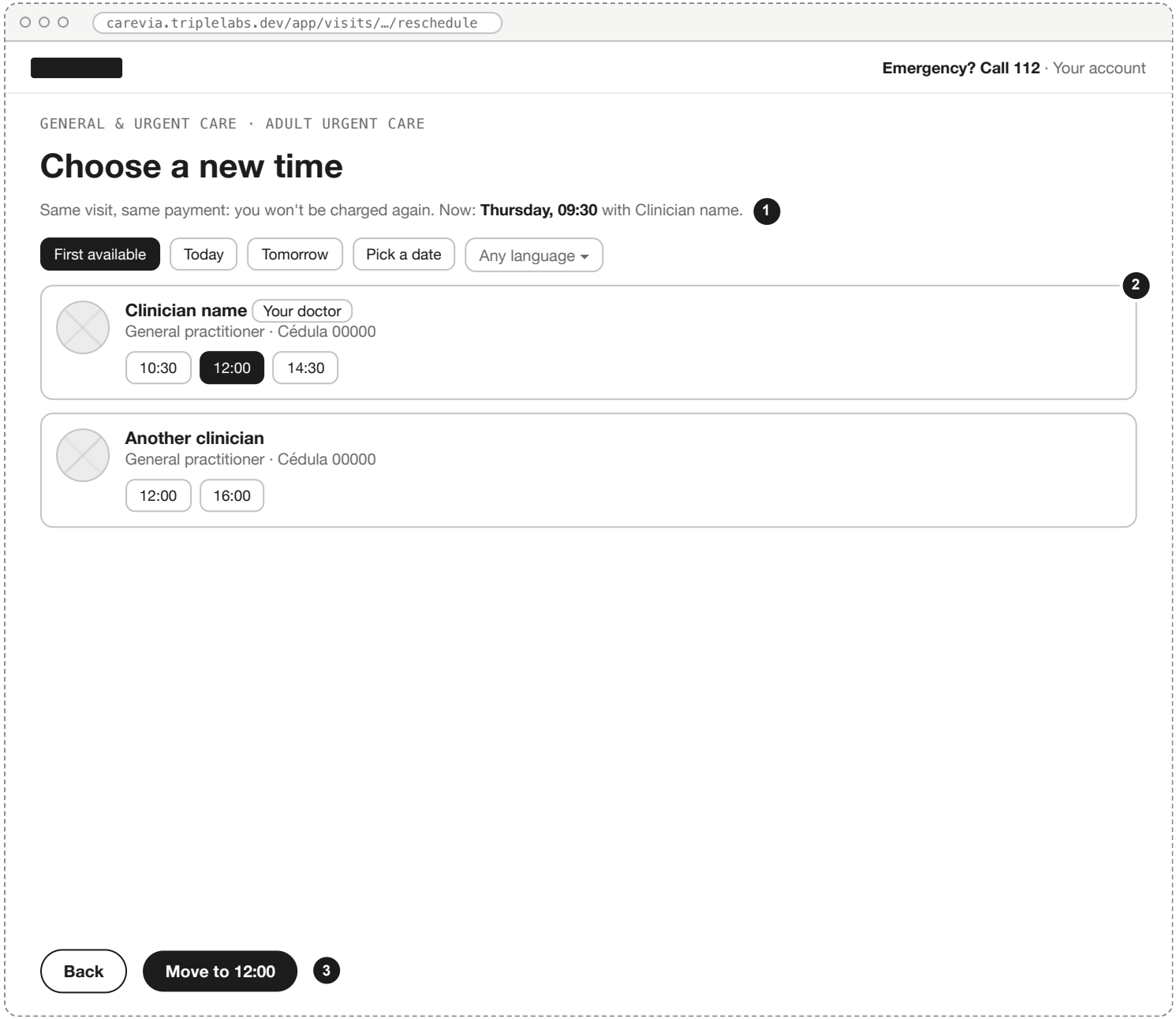


1. **Email and mobile, confirmed** NEW
   Both ticks are required before the waiting room. Carevia stores only the time they were confirmed.
2. **A companion** NEW
   Optional. A companion gets their own link, which can't control the room. The doctor sees that the patient asked to be accompanied.

Arrives here from: "Test camera and microphone" or "Join" on screen 08, once the room is about to open. Moving the visit to a new time asks again.

08c · Change the time (same appointment)

NEW SCREEN

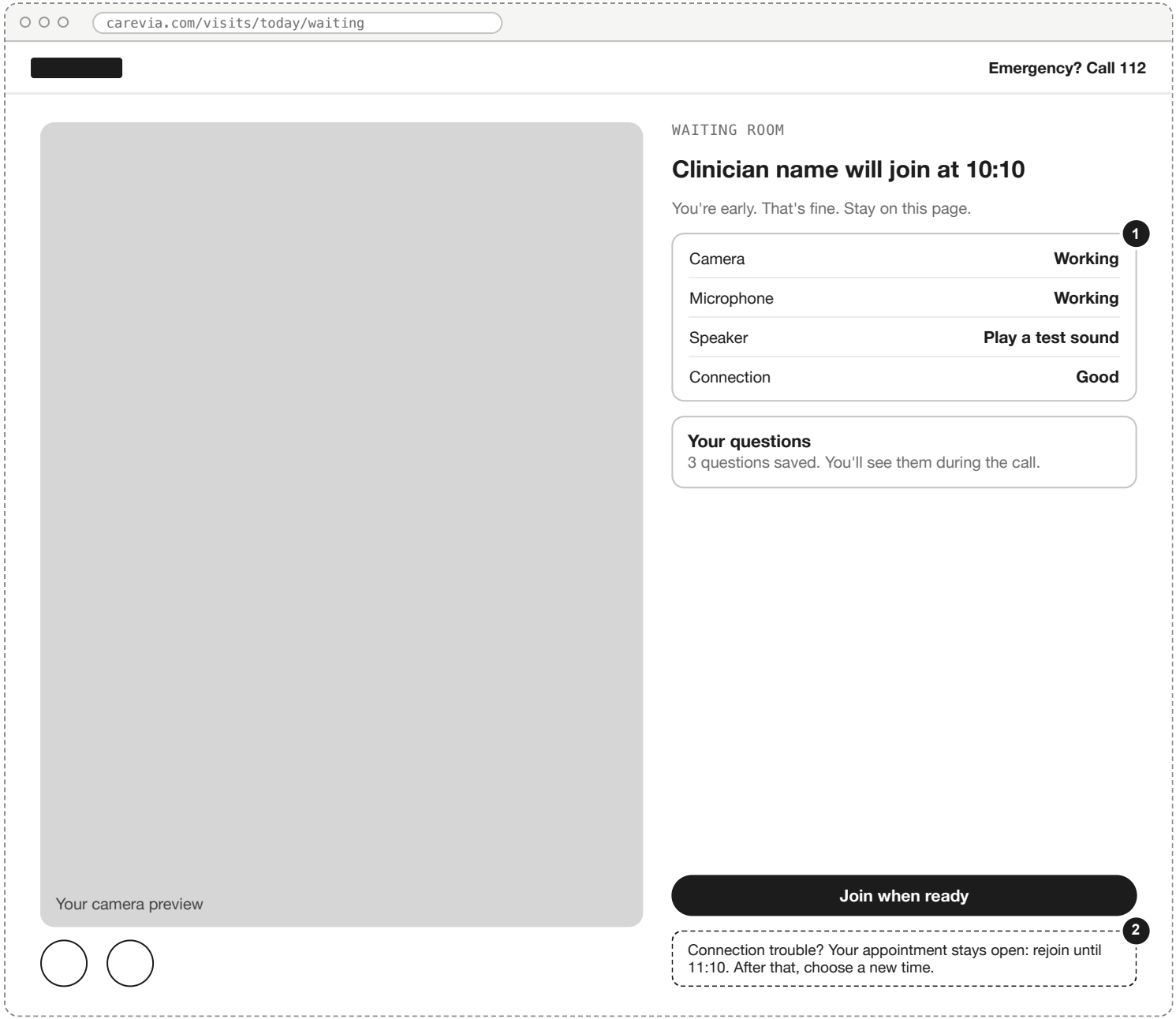


1. **The same appointment** NEW
   Moving keeps the consent and the payment. The old time is freed for someone else, and the confirmation email is resent with the new time.
2. **Your doctor first**
   The patient's own doctor is listed first; any doctor in the same area of care can also be chosen.
3. **Two ways in**
   "Change the time" on screen 08 before the visit, or "Choose a new time" on screen 11 after a call that dropped and wasn't rejoined.

Never a second booking. One person can't have two visits at the same time; the app refuses the clash.

09 · Waiting room

REVISED

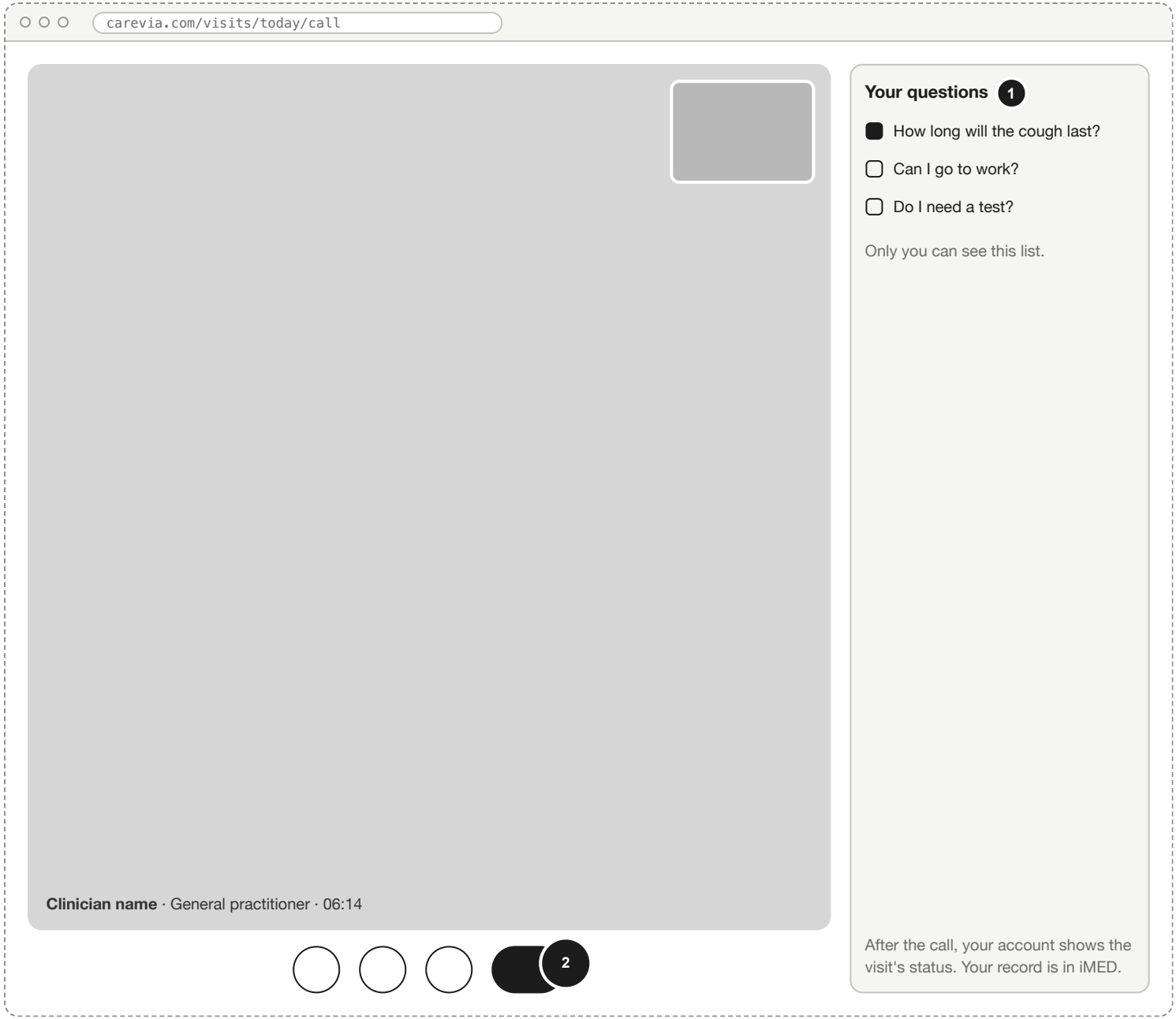


1. **Device check** NEW
   Clear "working" labels, not technical jargon.
2. **Safety net on screen** REVISED
   A dropped call keeps the same appointment: rejoin while the visit is open, otherwise move it to a new time. Never a second booking, never a second charge.

The most anxious moment of the journey. The screen's only jobs are to reassure, confirm the tech works, and show who is coming.

10 · The video consultation

NEW SCREEN

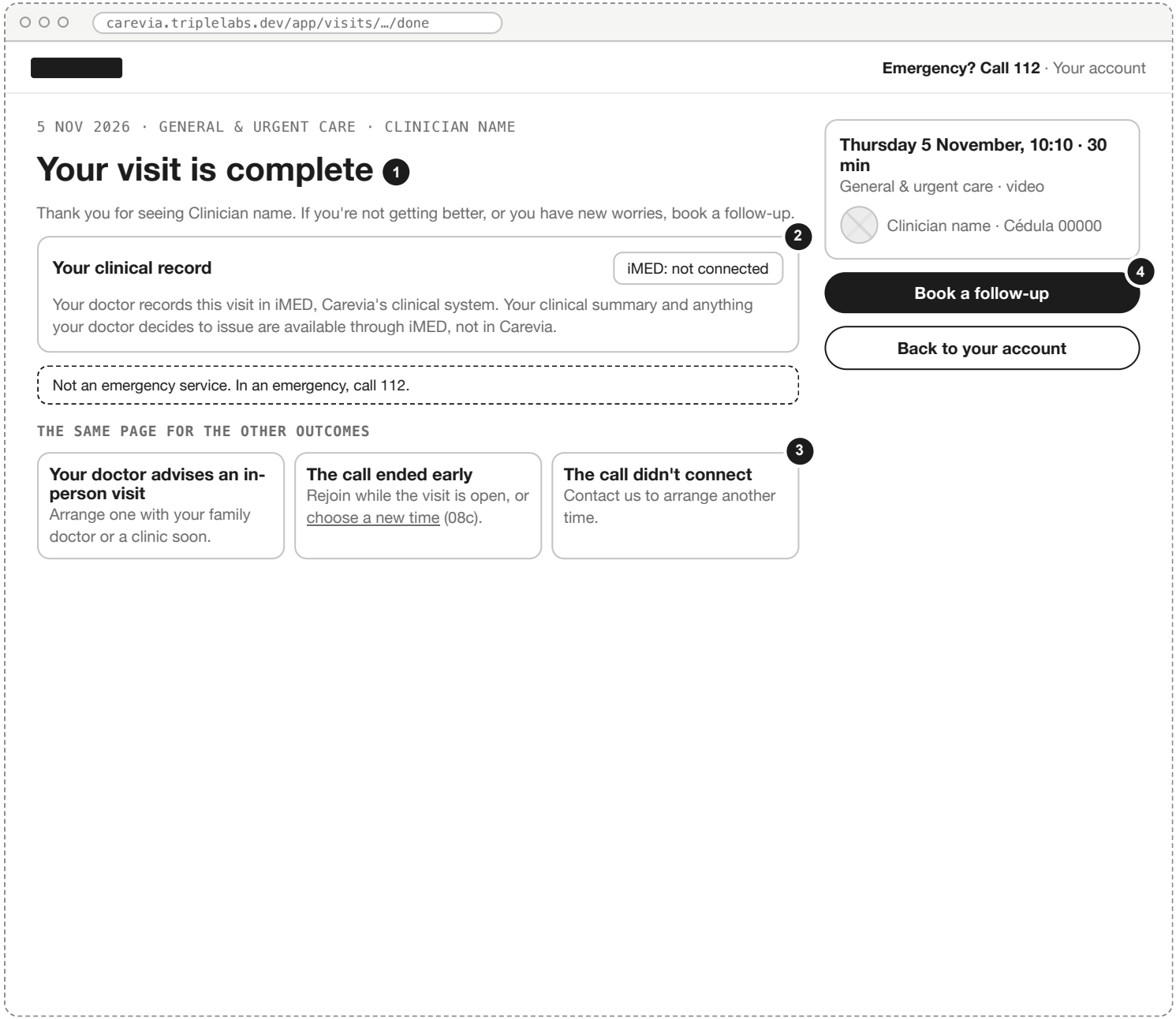


1. **Questions beside the video** NEW
   What the patient wrote while preparing, ticked off as they go. Nothing important is forgotten. Web only; on mobile the screen stays clear for the call.
2. **Four controls**
   Mute, camera, chat and end call. Nothing else.

"We listen." The screen is about the conversation, so everything else stays out of the way.

11 · After the visit: status and iMED

REVISED

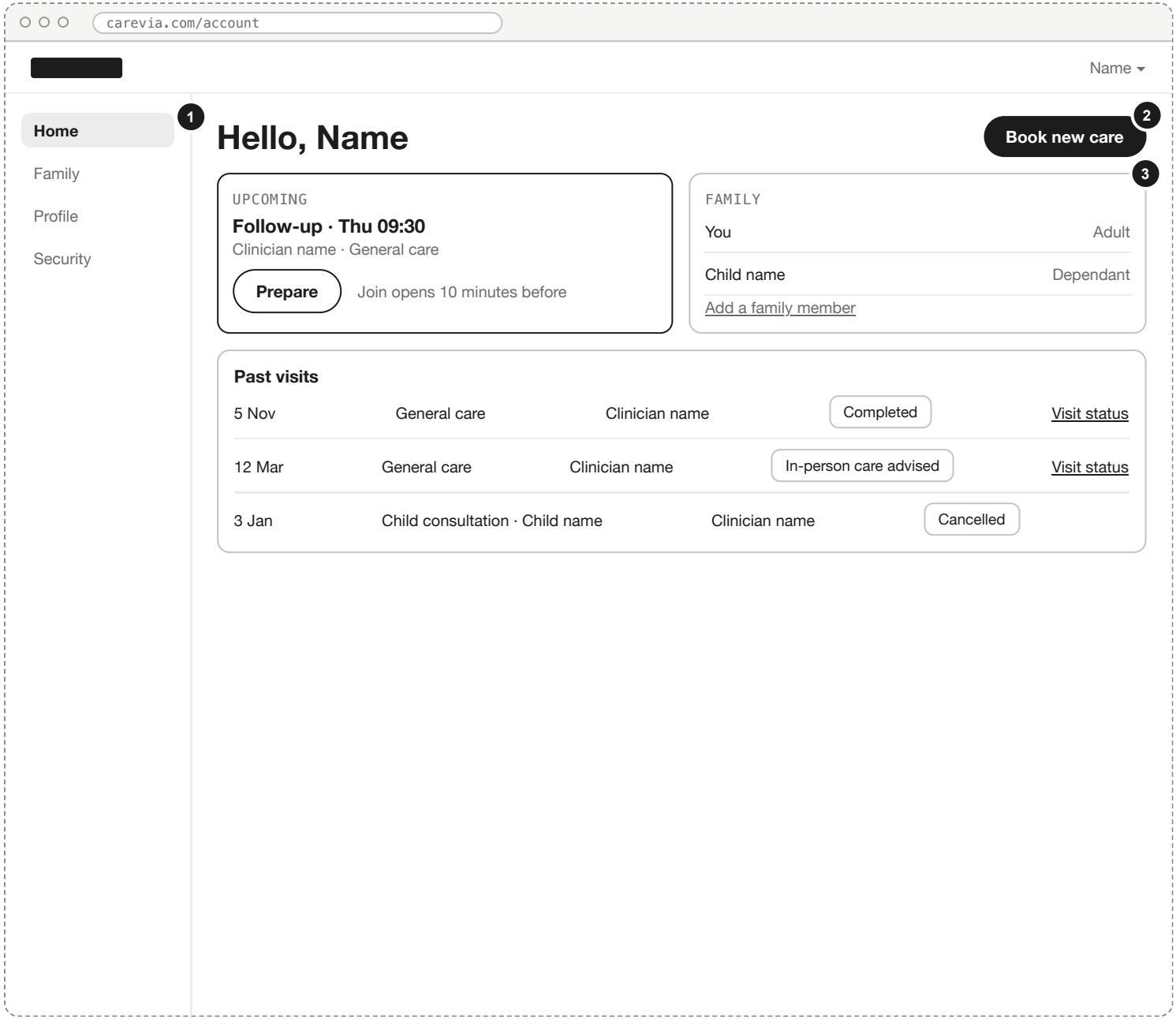


1. **A status, never a document** REVISED
   The doctor's outcome in plain words: completed, in-person care advised, ended early or didn't connect. No summary text, no next-steps list, no downloads.
2. **Where the record is** REVISED
   Prescriptions, sick-leave certificates (CIT), certificates, test requests and referrals are issued only in iMED. Carevia never generates, stores or shows them. "Not connected" becomes a live status when iMED is linked.
3. **Other outcomes**
   The heading and the next step change with the outcome. A dropped call keeps the same appointment.
4. **Follow-up in one click**
   Same doctor and area of care, chosen for you on step 1.

Why it changed: clinical documents live only in iMED, the certified system. Showing even a placeholder would break the rule.

12 · Account home

REVISED

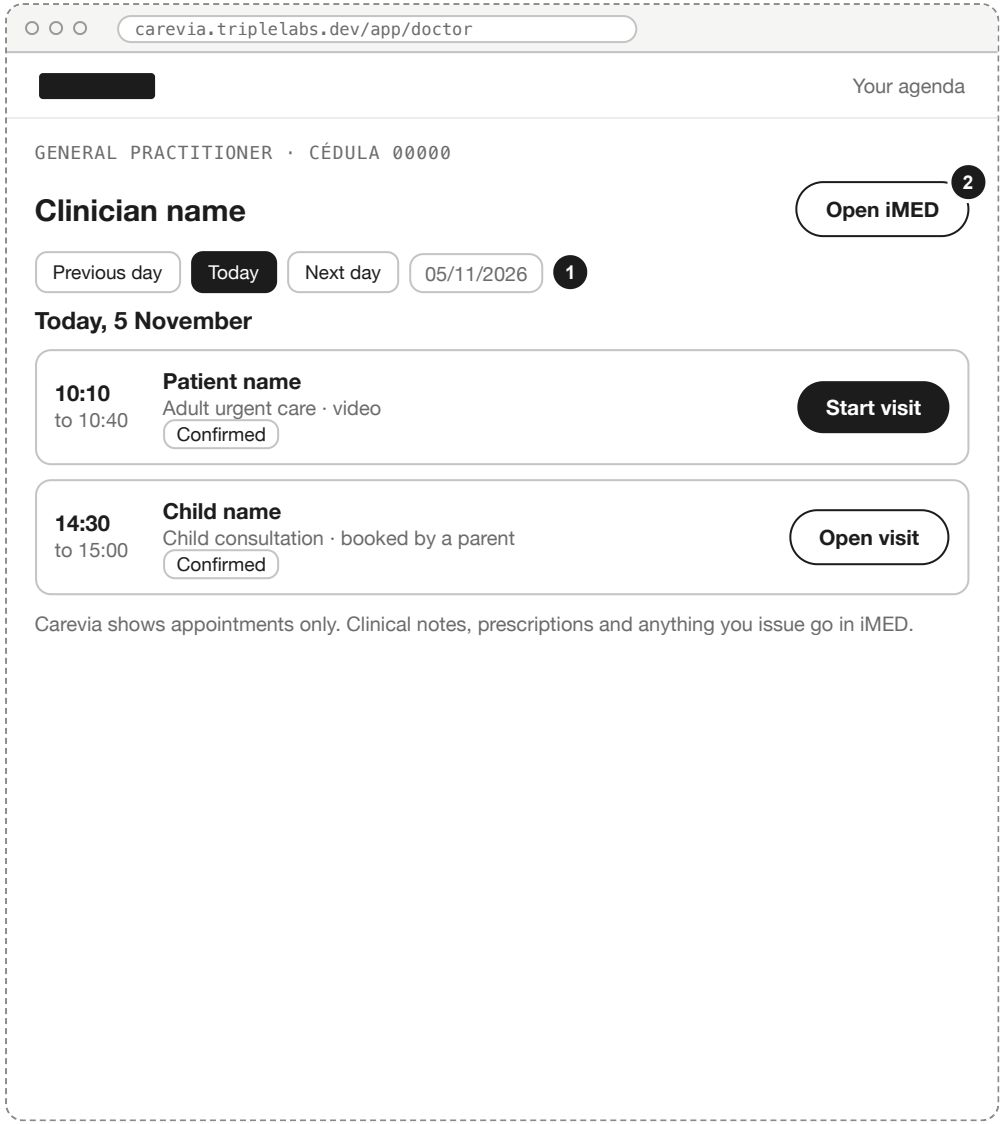


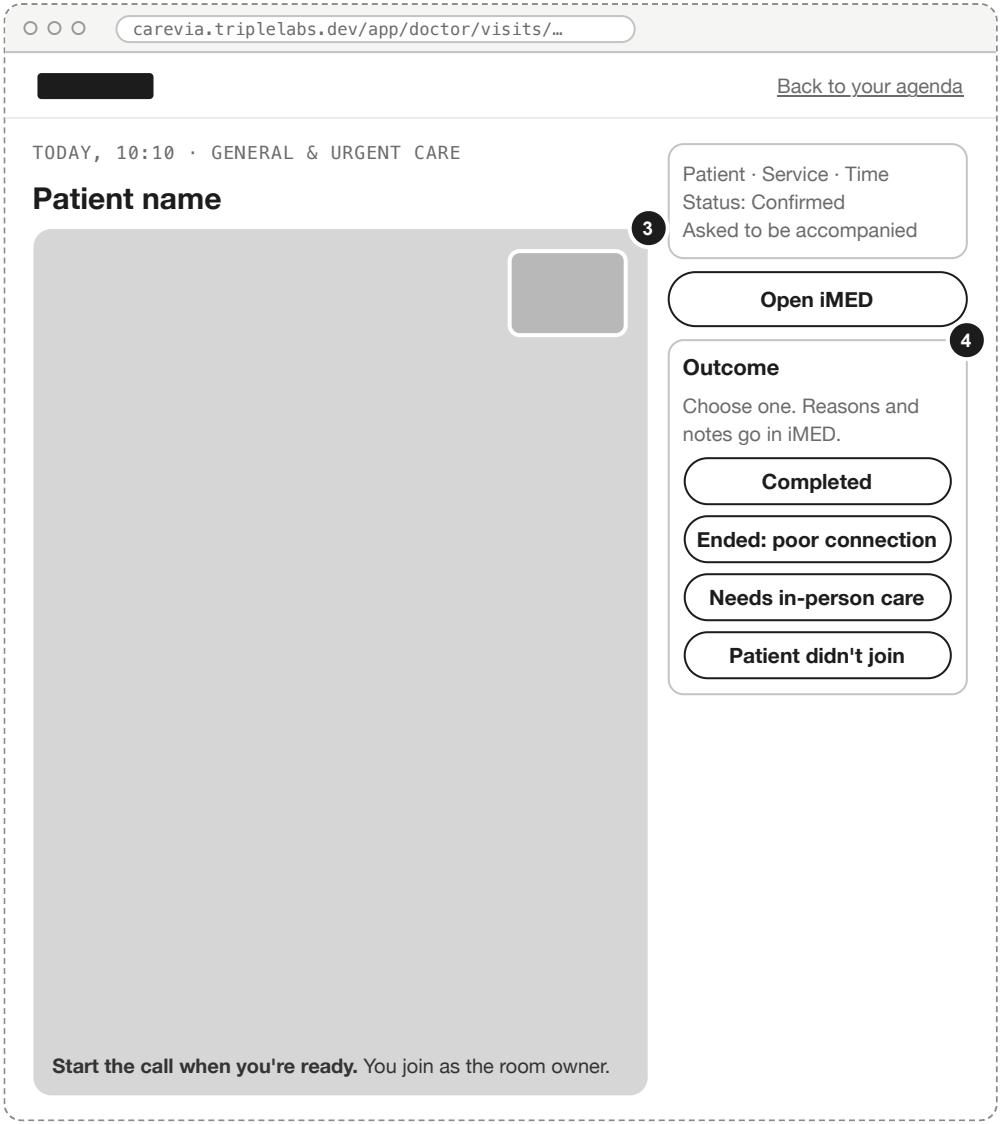
1. **Everything in one place** REVISED
   Visits and family. No Documents section: the clinical record and anything the doctor issues are in iMED.
2. **Book new care**
   Returning patients start booking from here, not from the homepage.
3. **Family** NEW
   Parents manage their children's visits from the same account.

The journey loops back: the next time this patient needs care, they start here.

13 · The doctor's agenda and visit room

NEW SCREENS





1 The day's visits
Time, patient's name, service and call state. No clinical fields, because Carevia has none.

2 iMED, one click away
The clinical record, prescriptions and certificates are written in iMED.

3 The doctor owns the room
A private room per visit. The doctor's link can admit and remove; the patient's and companion's can't. Never recorded.

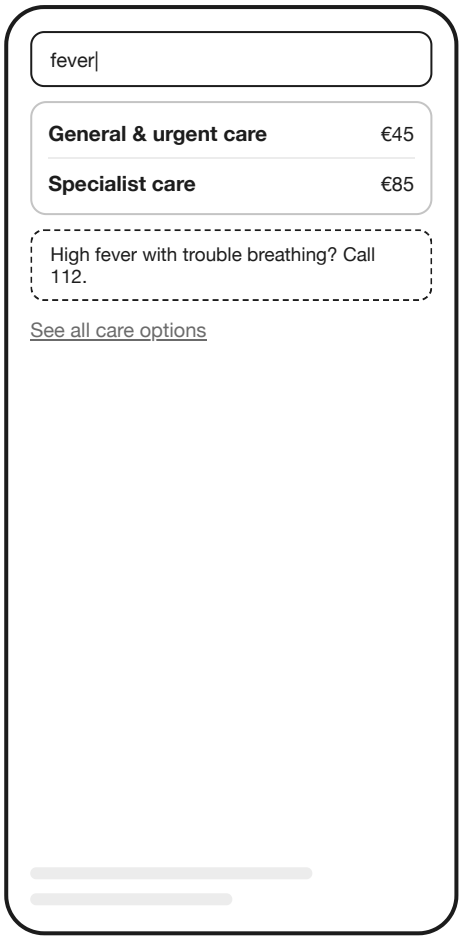
4 Four outcomes, no free text
Each asks to confirm. "Ended: poor connection" keeps the appointment open so the patient can rejoin or move it.

Mobile 01–03 · Discover

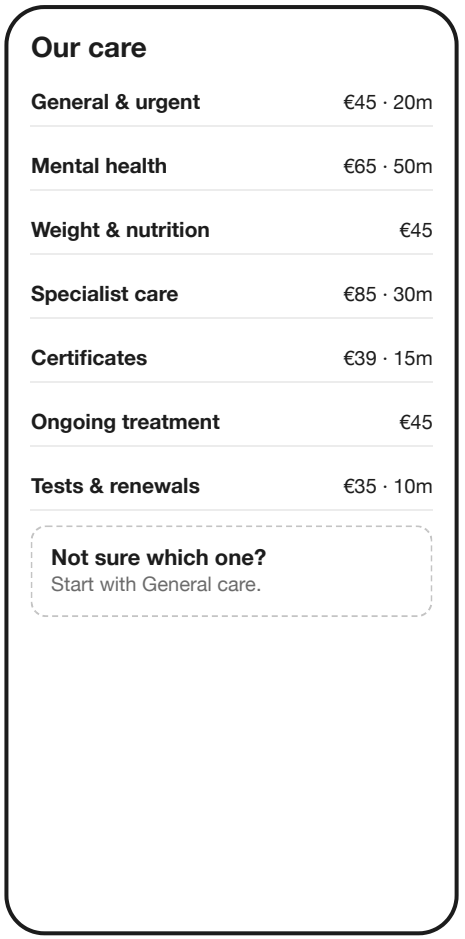
Exists New



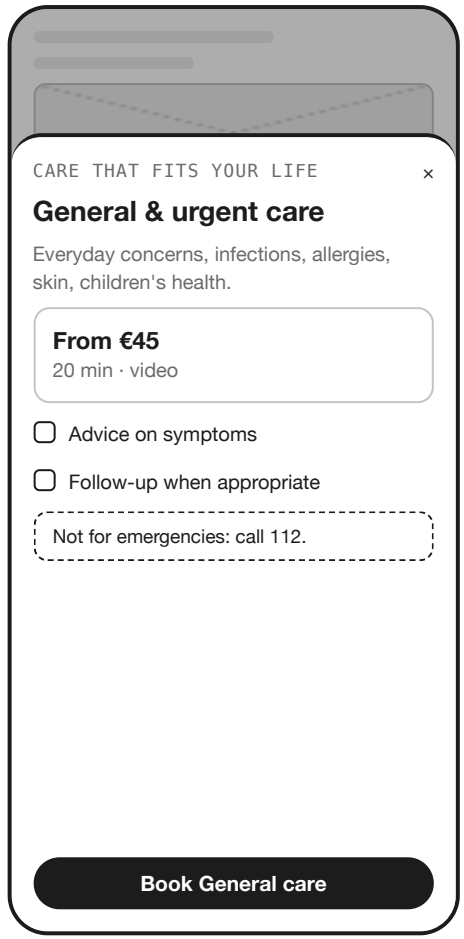
01
Homepage
Same as web. "Sign in" lives inside the menu.



01b
Search by symptom
Results open the service details.



02
Our care
A list, not a grid, for one-handed scrolling. New "Not sure?" card.

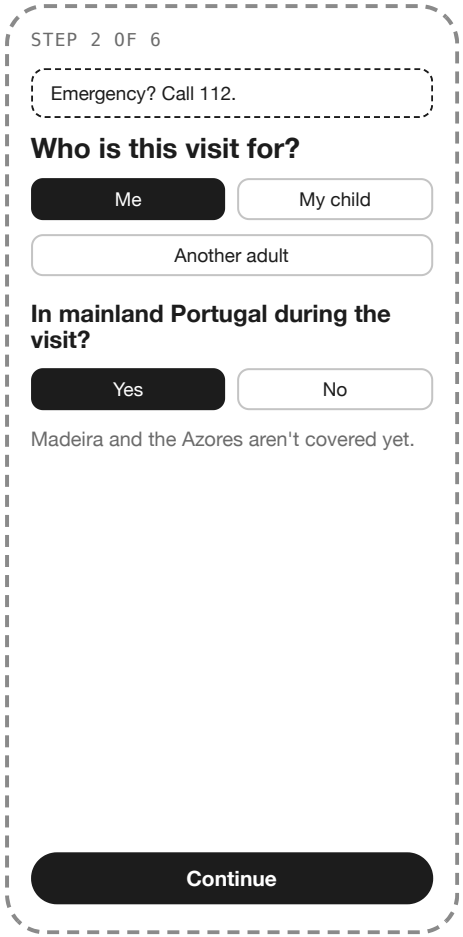


03
Service details
CHANGED
Slides up from the bottom. Button now starts booking with the service chosen.

Mobile 04–07 · Decide and book

STEPS 1 (CARE) AND 5 (CONSENT) LOOK LIKE THE WEB SCREENS

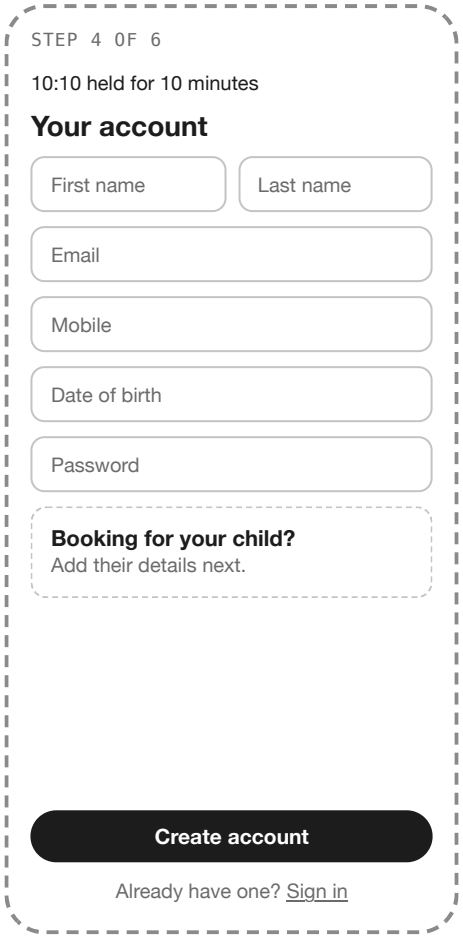
All new



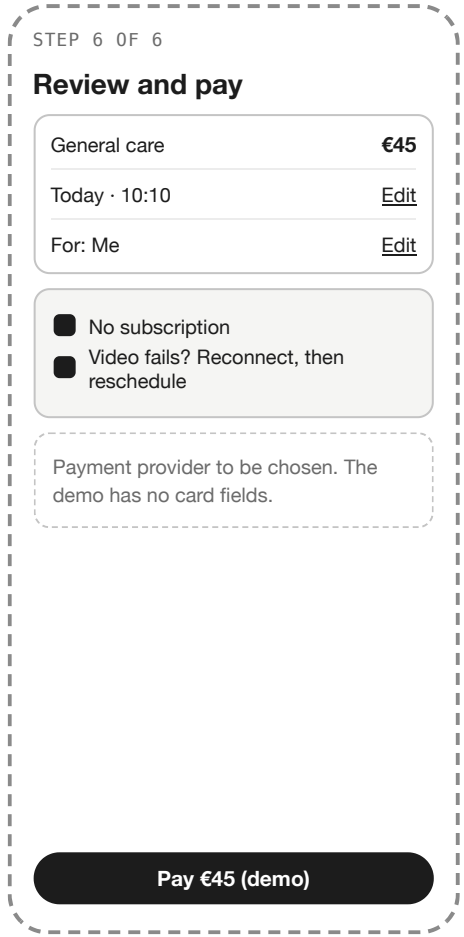
04
Who and where
Age and place checked before any details.



05
Clinician and time
"First available" sits first for speed.



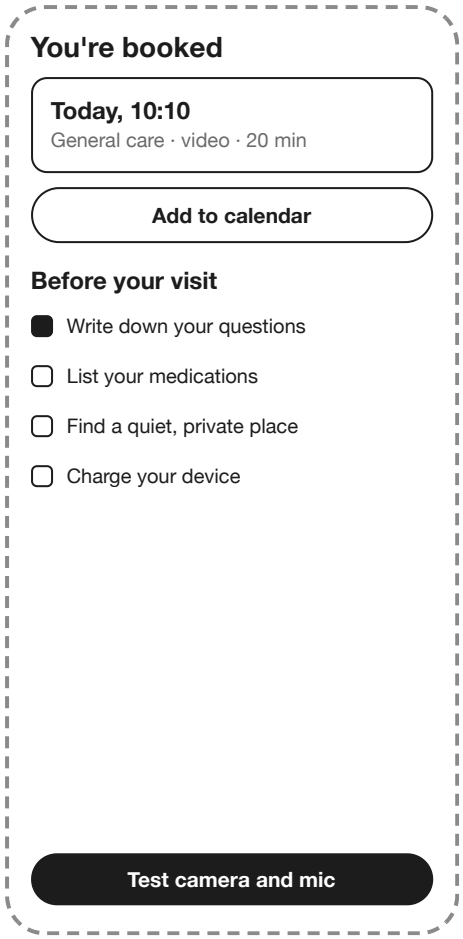
06
Your account
After the time is held, not before.



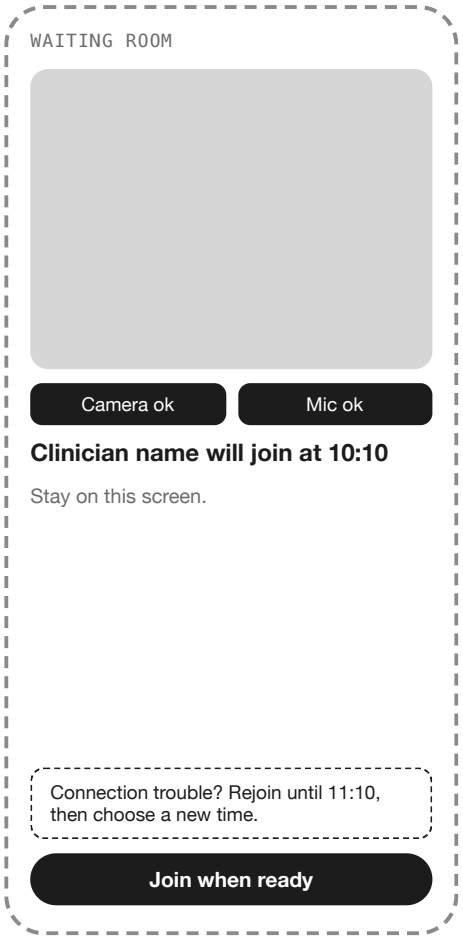
07
Review and pay
Reassurance beside the Pay button. Consent comes just before (step 5).

Mobile 08–10 · The visit

All new



08
Booked, and how to prepare
The preparation tips as a checklist.



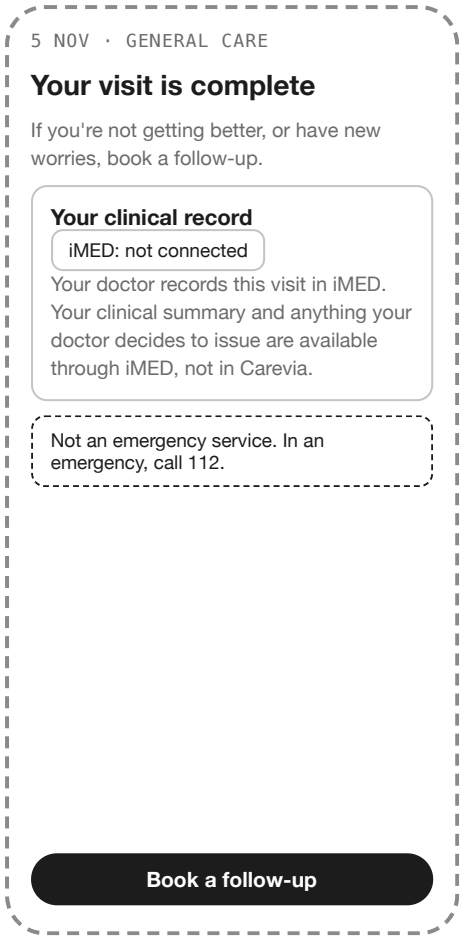
09
Waiting room
Device check, and the same appointment if the call drops.



10
Video consultation
Full screen for the conversation. Questions list one tap away.

Mobile 11–12 · After the visit

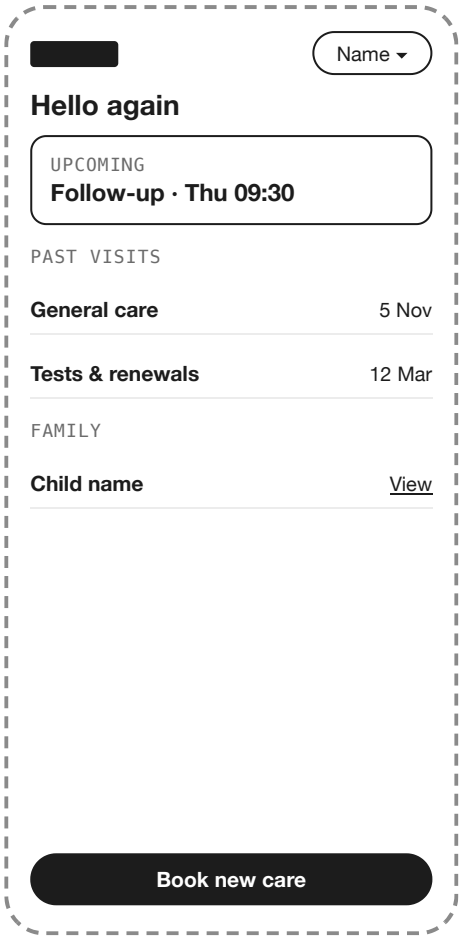
All new



11
After the visit

REVISED

A status and where the record is. Never a document.



12
Account home

Returning patients start here.

Why mobile matters most

Many patients will book from a phone, often while unwell. So on mobile:

- One decision per screen
- Main button always at the bottom, in thumb reach
- Big tap targets, at least 44 points
- Nothing during the call except the call

SECTION 6

When things don't go to plan

A good journey also handles the bad days. These four situations are designed into the screens above.

It's an emergency

Where: any screen.

"Call 112" shows on the service details, the search (for urgent-sounding words), booking steps 1 and 2, the waiting room and after the visit. Carevia never looks like an emergency service.

I can't use Carevia (place or age)

Where: booking step 2 (screen 04).

Checked before any personal details or payment. The patient sees a short, kind explanation and what to do instead. Under-18s are booked by a parent or guardian.

The video doesn't work

Where: waiting room or during the call (screens 09–10).

The same appointment stays open: rejoin until 60 minutes after the end. After that, or if the doctor records "ended: poor connection", the patient picks a new time for the same appointment (screen 08c). No second booking and no second charge.

It can't be treated online

Where: during the call, then the visit status (screen 11).

The doctor records "needs in-person care". The status says so and asks the patient to arrange an in-person visit. Any referral is issued in iMED, never in Carevia.

SECTION 7

Questions for you, and next steps

Your answers will shape the next round of design. None of these block the journey above, but each changes a screen.

Questions

1. Choosing a clinician. Should patients pick a specific clinician, or only a time ("first available")? We show both. Screen 05

2. Holding a time. Is 10 minutes right while a new patient creates an account? Screen 06

3. Payment. Which payment provider? Do we need insurance, vouchers or company billing? Screen 07

4. Cancelling. Until when can a patient cancel or reschedule for free? Screens 08, 12

5. Place and language. *Answered:* mainland Portugal at launch; the demo is in English and Portuguese is a launch requirement. Screens 04, 05

6. Emergency wording. Can your clinical team approve the emergency messages and the symptom words that trigger them? Screens 01b, 04, 09

7. Recordings. *Answered:* never in this build; it would need its own consent. Screens 06b, 10

8. Service names (decided). Services named after a document are now named as assessments, for example “Sick leave assessment”, “Illness assessment” and “Treatment renewal review”, so no name reads as a promise. Screens 02, 03b

9. Reminders. Email and SMS reminders need a sender and a data-processing agreement. Which provider? Screen 08

Suggested next steps

1. Review this document
Mark anything to add, remove or reorder. Answer the questions on the left.

2. Mid-fidelity designs
Real wording, Carevia's colours and type, for the booking steps first (screens 03–07), since that's where people drop off.

3. Clickable prototype
Test it with five real patients before anything is built.